Anastrozole · daily hormone therapy

Take anastrozole once daily continuously. There is no planned week off. The treatment setting determines the intended duration.

Hormone therapy · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Joints and muscles

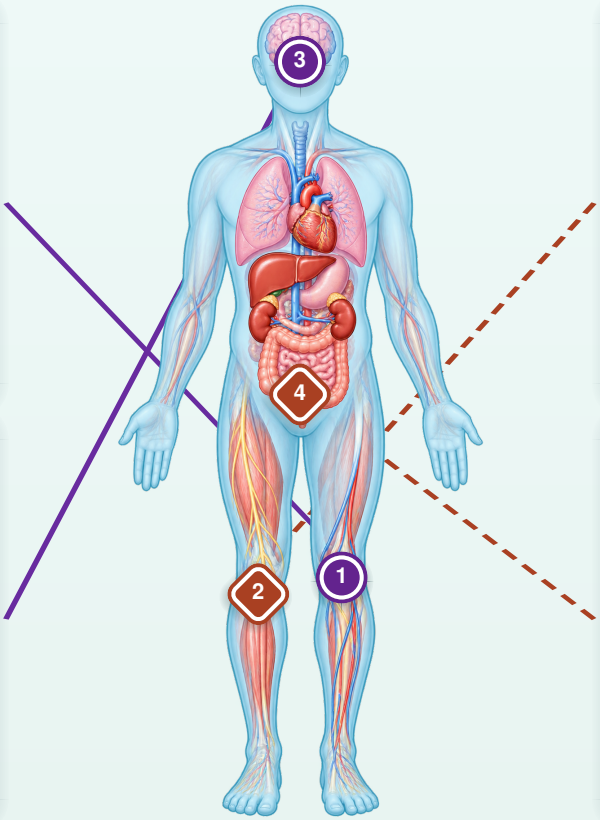
Anastrozole

Pain or stiffness can affect activity; report persistent or limiting symptoms.

2 Bone strength

Anastrozole

Bone loss may be silent. Your risk assessment and bone-density plan matter.



3 Hot flushes / sleep

Anastrozole

Warm flushes and disrupted sleep can be treated; tell the team.

4 Vaginal / sexual symptoms

Anastrozole

Dryness or discomfort deserves support. Ask before using hormone-containing treatments.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Report persistent limiting joint pain, new tendon pain or swelling, a possible fracture, jaundice or new vaginal bleeding. Sudden loss of tendon function needs prompt assessment. Severe allergic symptoms, chest pain or major breathing difficulty require urgent help. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

YOUR PRACTICAL TREATMENT PLAN

Know the next step.

Taking treatment

Take anastrozole once daily continuously. There is no planned week off. The treatment setting determines the intended duration.

Swallow the tablet whole with water, with or without food. Do not replace a missed or vomited dose; resume the next scheduled dose.

Checks and follow-up

full blood count (FBC), renal and liver profile. Confirm menopause; hormone tests may be needed if it is uncertain. Assess fracture risk and baseline bone density, particularly for adjuvant aromatase inhibitor (AI) treatment. The NICE adjuvant baseline-dual-energy X-ray absorptiometry (bone density) (DXA) recommendation has an exception for patients already receiving adjuvant bisphosphonates. Assess symptoms, adherence and bone health at reviews. Repeat lipids and other blood tests when clinically indicated. Bone-density timing follows baseline results and fracture risk.

The whole-treatment map distinguishes early-breast follow-up from advanced-disease response assessment.

Support between reviews

Symptoms and daily life

Tell the team if joint pain, hot flushes, vaginal dryness or sleep disturbance affects daily life. Bone thinning may cause no early symptoms, so keep the agreed bone-health reviews.

Support medicines

No routine antiemetic, steroid premedication or granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) is required by this endocrine monotherapy. Ask for individual symptom support.

Tests and appointments

Ask who books each check, when it is due and who reviews it. Tell the team if an expected appointment has not arrived.

Emergency: 112 / 999. Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

Contact your treating team promptly: Report persistent limiting joint pain, new tendon pain or swelling, a possible fracture, jaundice or new vaginal bleeding. Sudden loss of tendon function needs prompt assessment. Severe allergic symptoms, chest pain or major breathing difficulty require urgent help.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your agreed out-of-hours pathway

Other hospitals: use the contacts your own team provides.

Your complete treatment reference

https://sactcheck.com/patient/00254/

Public information only. No personal information is encoded.